

Terminology

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Therapies

FAQs

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Cases

Gender affirming care in adults



Terminology

How to ask

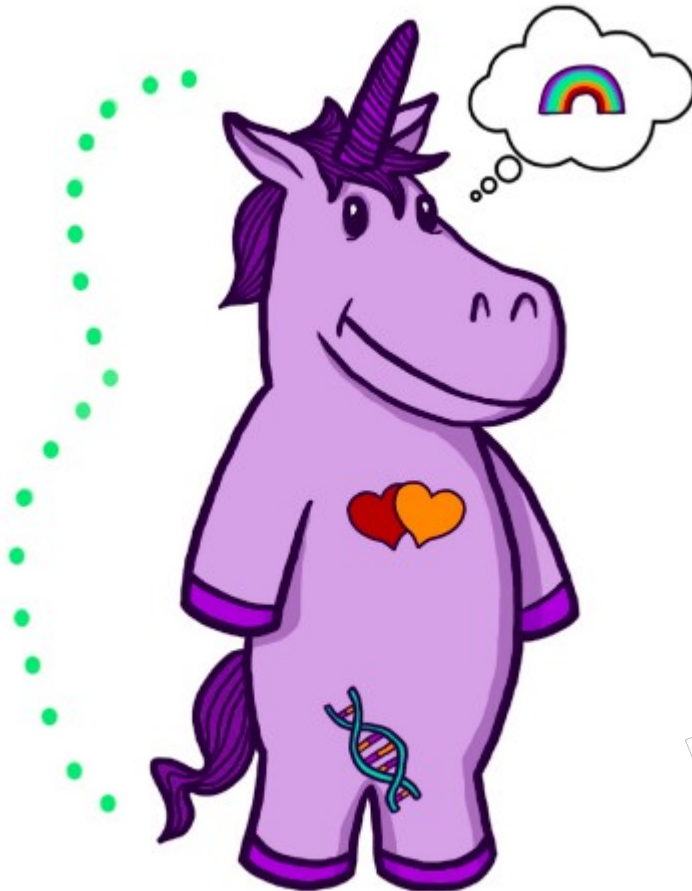
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Gender identity

Gender diverse
Cisgender
Transgender
Nonbinary
Genderfluid



Gender expression



Sex assigned at birth

Assigned male at birth (AMAB)
Assigned female at birth (AFAB)
Differences of sexual development (DSD, formerly intersex)



Sexual orientation

Physical attraction
Emotional attraction

Epidemiology of gender diversity

1.0% millennials
2% gen Z
> 2.6 million
Americans

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Name?

What name do you go by? (Avoid 'deadnaming')

Pronouns?

What pronouns do you use?

He/him/his

She/her/hers

They/them/theirs

Alternate - Ze, Zim, Zirs, etc.

Gender?

How would you describe your current gender identity?

Sex?

What sex was assigned to you at birth? (What was marked on your birth certificate?)

Organ inventory?

What organs do you have or what you've had removed as a part of your transition?
Do you still have a cervix or not? And is that a word you're comfortable with?

Mis-naming and Mis-gendering

It happens!

1. Recognize
2. Apologize
3. Commit to change
4. Move on

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Diagnosing Gender Dysphoria

- Diagnosis of gender dysphoria (can be made by a generalist!)
- Gender incongruence is not pathological
- Mental health considerations



DSM criteria for gender dysphoria



Informed Consent

- Capacity
- Assess for contraindications
- Expectations, changes, and side effects
- Fertility counseling



Treatment Options

- Patient goals
- Gender affirming hormone therapy
- Gender affirming surgery
- Vocal therapy
- Mental health



Monitoring

- Role of medical home
- Medical monitoring
- Interdisciplinary care

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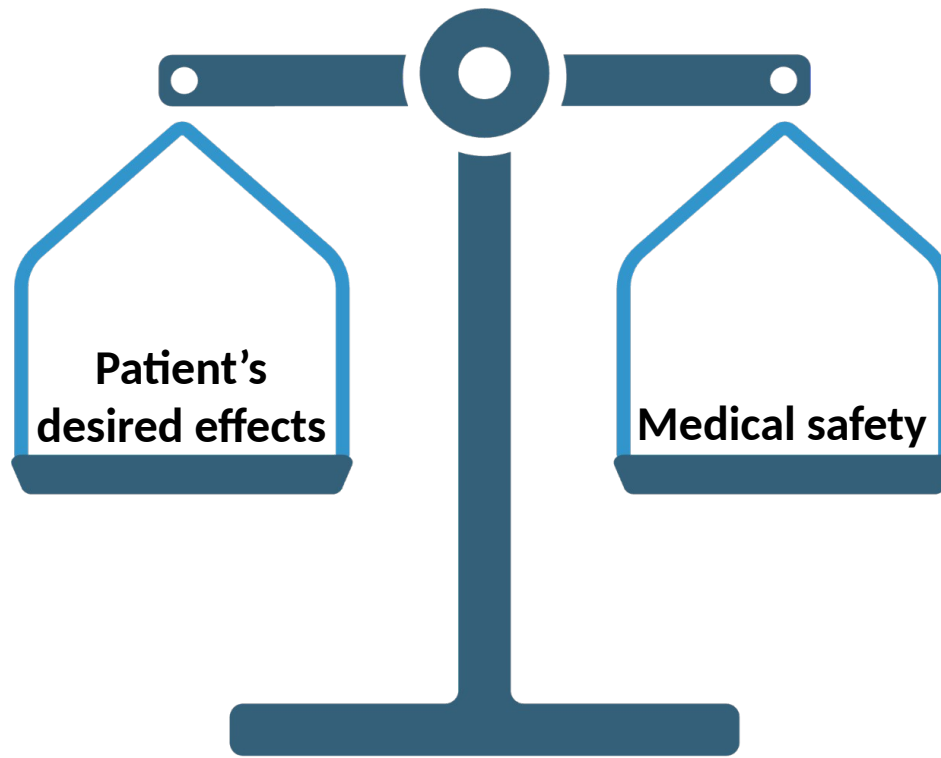
Feminizing

Masculinizing

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Feminizing Hormones

Estrogen

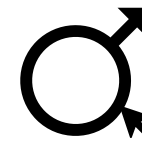
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Anti-androgen

17 β -estradiol
(bio-identical)

Spironolactone
5- α reductase inh**
GnRH Agonists

** Generally used with spiro



Masculinizing Hormones

Testosterone

Bio-identical testosterone

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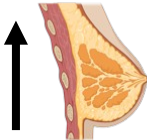
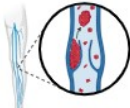
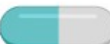
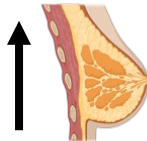
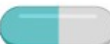
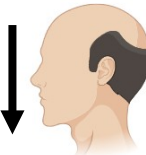
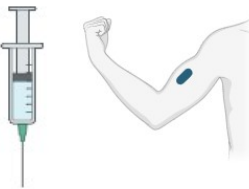
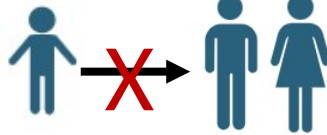
Dosing regimens

Estradiol

Spironolactone

5- α reductase inhibitors

GnRH Agonists

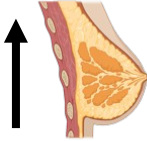
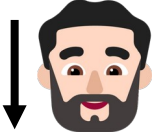
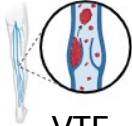
Formulations	Desired Effects	Contraindications & Side Effects	Monitoring
	 Breast & adipose  Body hair	 VTE  Estrogen dependent cancers	Total Estradiol*, Total Testosterone, LFTs & lipids 3mo & 6mo...Q1yr
	 Breast & adipose  Body hair	 K  Libido	Cr & K 3mo & 6mo...Q1yr
	 Balding	 Orthostasis  Libido	None
	 Block 2 ^o characteristics	 Osteoporosis  CV events	Estradiol, Testosterone, & LH/FSH 3mo & 6mo...Q1yr

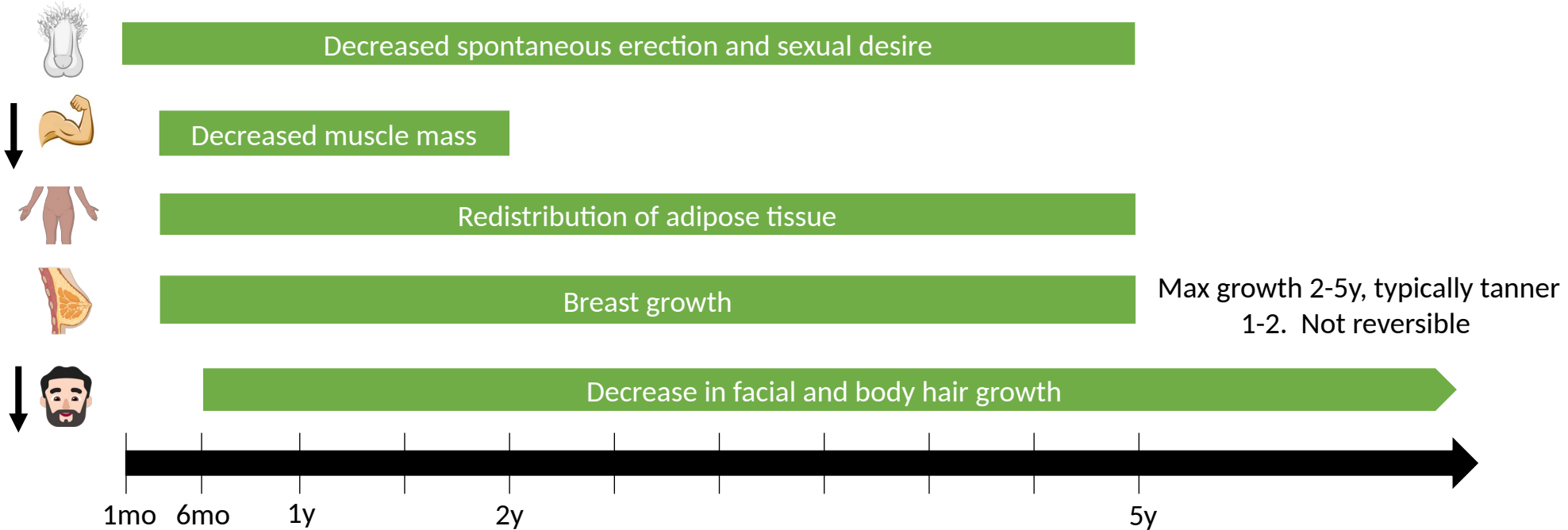
*Goal = 100-200 pg/mL

Timeline of effects

- Terminology
- Evaluation
- Therapies
- Feminizing**
- Masculinizing
- FAQs
- Resources
- Cases

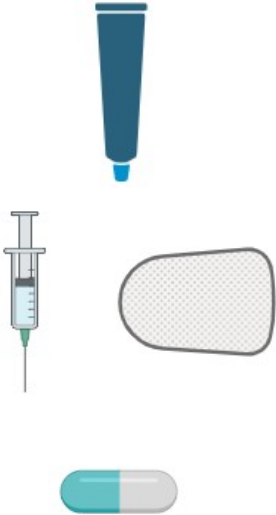
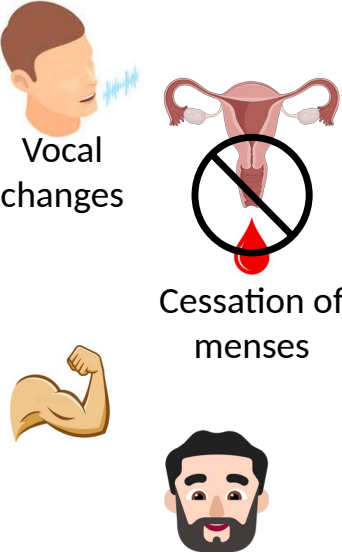
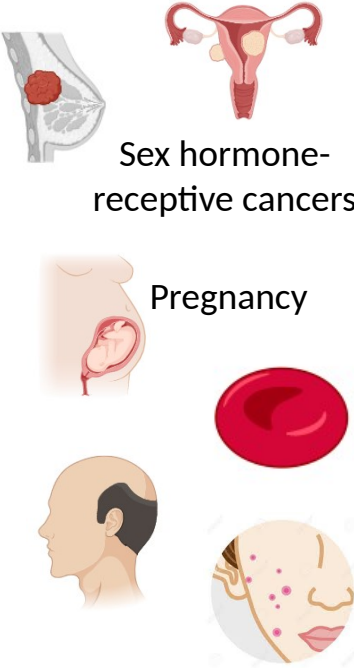
Estrogen

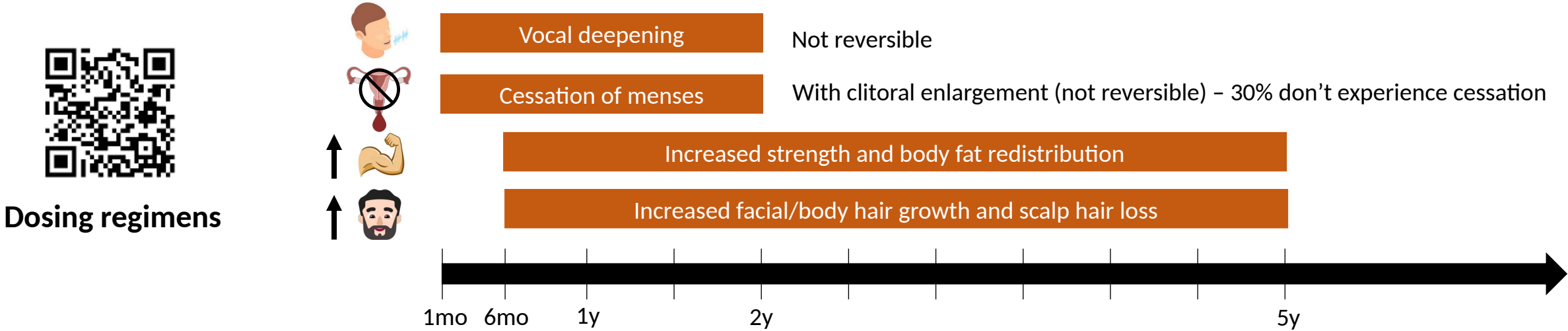
Formulations	Desired Effects	Contraindications & Side Effects	Monitoring
	 Breast & adipose  Body hair	 VTE  Estrogen dependent cancers	Total Estradiol*, Total Testosterone, LFTs & lipids 3mo & 6mo...Q1yr



- Terminology
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Testosterone

Formulations	Desired Effects	Contraindications	Monitoring
	 Vocal changes Cessation of menses	 Sex hormone-receptive cancers Pregnancy	Total Testosterone (consider estradiol) 3mo & 6mo...Q1yr <u>Goal testosterone level between doses:</u> 400-700 ng/dL



Terminology

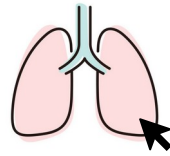
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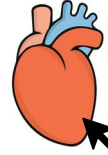
Resources

Cases



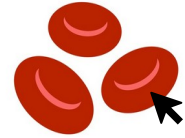
DVT/PE on estradiol?

- Risk-benefit
- Patch is lowest risk
- Consider anticoagulation



CV risk on estradiol or testosterone?

- Risk-benefit
- Slight elevated CV risk
- USPSTF screening guidelines
- See STRONG study in guide under objective four



Polycythemia on testosterone?

- Rule out other causes
- Decrease dose, increase frequency, change injections -> topical (**goal to decrease testosterone variability**)
- Rule out OSA



Stopping hormones perioperatively?

- Do not hold, with exception of estradiol with risk factors



Cancer screening?

- Based on organs present



Cost & access for GAHT?

- State Medicaid variable, Medicare should cover
- Surgery often self-pay
- Know local legislation
- See LGBT map on next slide

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Resources in gender affirming care



UCSF Transgender
Care Guidelines



World Professional
Association for Transgender
Care Guidelines (Version 8,
Published 2022)



Gender Affirming
Hormone Therapy
Quick Guide



Fenway LGBTQIA+
Health Modules
(also available for
CME)



LGBTmap.org: guide to policies in each state

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Case 1

A 24-year-old person who was assigned female at birth (AFAB) who has been on masculinizing therapy for 6 months presents to the hospital for 2 months of generalized fatigue as well as two weeks of fevers, chills, and dysuria.

Role play an initial interaction with this patient using appropriate terminology.

An appropriate role play will contain:

1. Asking preferred name, pronouns
2. Discussing gender, gender identity, and sex in a patient-centered way. (See Terminology, slide two)

Is an organ inventory appropriate in this case?
If so, practice performing one.

Yes, a focused organ inventory is appropriate. Correct organ inventory should include:

1. Asking about patient preference in referring to organs.
2. Using **neutral language** when appropriate
3. Assessing for the presence of relevant organs, which in this case is suggested to include vagina, uterus, penis (constructed), urethra (history of surgery).

Create a list of expected anatomic and physiological changes at six months, and whether these changes impact differential.

Changes at six months of masculinizing therapy likely include:

Increased production of skin oil/acne; vocal deepening; menstrual interruption; possible increase in strength and male secondary sex body hair, clitoral enlargement, vaginal atrophy.

We also suggest a discussion about fertility (the patient could still become pregnant) and sexual history (recent male/female partners, protection).

Which medications may be used to provide gender affirming hormone therapy to this patient, and what laboratory levels may be appropriate at this time.

Medications: Testosterone.

Labs: CBC to check for polycythemia (no clear role for total testosterone level based on presenting symptoms).

Should gender affirming medication should be held during this hospitalization?

Testosterone should **not** be stopped while this patient is hospitalized unless clear signs/symptoms of polycythemia.

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Case 2

A 38-year-old individual presents to outpatient clinic to discuss initiation of feminizing hormones. Their sex assigned at birth is male.

Role play an initial interaction with this patient using appropriate terminology.

An appropriate role play will contain:

1. Asking preferred name, pronouns
2. Discussing gender, gender identity, and sex in a patient-centered way. (See objective one)

Discuss considerations when evaluating a patient for start on gender affirming hormone therapy or gender affirming surgery.

Considerations include:

1. Diagnosis gender dysphoria
2. Capacity for consent
3. Evaluating for contraindications
4. Expectations/goals/side effects
5. Fertility counseling

Explain to the patient which medications are available for feminizing hormone therapy and the timeline of anticipated changes.

Estradiol, typically plus one or more anti-androgens.

A patient can expect to see decrease in strength, body fat redistribution, breast bud development, and often erectile dysfunction at three months; facial and body hair often decreases at about six months.

Create a list of possible side effects for the patient and how these side effects may present clinically.

- **DVT/PE:** unilateral swelling/cord or as sudden shortness of breath/elevated heart rate/chest pain
- **Spironolactone:** increased potassium (typically asymptomatic unless extreme), increased urination, and sometimes orthostasis
- **Alpha reductase inhibitors:** rarely orthostasis, sometimes decrease in libido
- All medications can cause erectile dysfunction

Counsel the patient on whether feminizing hormones have long term impacts on cardiovascular or bone health.

See the STRONG study presenter guide, objective four.

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UCSF estrogen in VTE
guidelines

Case 3

Rain is a 35 y.o. trans female (tF) non-binary person (they/them) on oral estradiol therapy for two years who presents with leg pain and swelling. On ultrasound they are found to have a deep vein thrombosis (DVT) extending through the right popliteal and femoral veins. There is no chest pain or hemodynamic instability.

Discuss the benefits and risks of stopping hormone therapy to this patient.

Benefits and risks should be discussed in a patient-centered way.

- Principal risk of continuing estradiol in this patient is extension of the DVT or conversion into a PE, which may be life threatening.
- However, **psychological and medical risks of stopping gender affirming hormone therapy (depression, suicide) may also be profound and should be weighed.**

Create a plan for moving forward if the patient chooses to continue gender affirming hormone therapy (GAHT).

If in alignment with patient goals to continue estradiol, consider the following interventions:

- Switch to transdermal estrogen
- Utilize therapeutic dose anticoagulation (see QR for UCSF guidelines, figures 1-3: estrogen in VTE)
- If symptomatic PE or hemodynamic instability, recommend stop estradiol and consider restart after resolution of PE symptoms

2024–2025 Currency & U.S. Policy Context

Standing clinical guidelines UNCHANGED for adults: WPATH Standards of Care v8 (2022) and Endocrine Society Clinical Practice Guideline (2017) remain the governing references; hormone regimens and targets in this talk are current.

U.S. policy/access changes since publication (affect ACCESS, not the adult clinical standard; scoped largely to patients <19):

Executive Order 14187 (Jan 28, 2025) directs the federal government not to support gender-affirming care for individuals under age 19.

CMS letters/proposed rules (2025; Federal Register Dec 19, 2025) restrict federal Medicaid/CHIP funding for gender-affirming care in minors.

HHS rescinded enforcement of ACA §1557 nondiscrimination protections for transgender patients (Feb 2025) — coverage denials face low federal enforcement risk.

Several measures are subject to ongoing litigation/injunctions. Verify current state and federal law and payer coverage before counseling — this is a rapidly evolving area.

Take-Home Points

1. Lead with patient-centered language: ask name, pronouns, gender identity, sex assigned at birth, and a focused organ inventory. If you misgender: recognize–apologize–commit–move on.
2. A generalist can diagnose gender dysphoria and start GAHT via informed consent (capacity, contraindications, expectations, fertility counseling). Gender incongruence is NOT a mental illness.
3. Feminizing = 17β -estradiol (goal 100–200 pg/mL) $\pm$ anti-androgen (spironolactone); masculinizing = bioidentical testosterone alone (goal trough 400–700 ng/dL). Titrate to desired effects vs side effects.
4. Know the drug–risk pairs: estrogen→VTE (transdermal lowest risk), spironolactone→hyperkalemia, testosterone→erythrocytosis. Don't hold hormones when admitted (except estradiol with VTE risk). Cancer-screen by organs present.

Attribution, License & Optimization Notes

Original: “Gender Affirming Care,” TeachIM.org. Authors: Brian Adams MD, Helene Hedian MD, Rita Lee MD; exec. editor Brandon Fainstad MD. Published May 2024 (page updated Aug 2024).

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Optimization (June 2026):

Corrections (surgical, animations preserved): epidemiology figure “1.2% millennials” → “1.0%” (matches lesson’s own teaching script / Gallup 2021); “17-estradiol” → “17 β -estradiol” (correct chemical name).

Companion GAHT learner handout corrected: “Maculinizing” → Masculinizing, “FSH/LSH” → FSH/LH, “USPTF” → USPSTF (filename preserved).

Added: 2024–25 currency/policy slide (above); HTML package with pre-work, ASK points, 5 MCQs, 17 Anki cards, EMR dotphrase, one-pager.

All 13 original interactive/click-to-reveal slides preserved. Human-review: speaker notes on slides 6–8 contain unrelated contraception/IUD text carried over from another lesson (non-projected) — left unedited.